**Mindfulness: Fulfilling the Promise at Last, with a Relational View**

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[Workshop begins with a guided meditation and dialogue]

## Introduction

Professionally, I live within two different communities of practice, the social/relational constructionist one gathered here now, and the mindfulness-based one, to which I remain attached, although more and more ambivalently. That ambivalence is where I'd like to begin exploring with you today.

I joined in each of these communities at the same time, at about the turn of the century. It seemed to me then that the potential of both to bring changes to the health care system, and, particularly, to the mental health care system, was significant. Both had a critical edge, and were developing practices that offered greater freedom to patients who were not only facing suffering in their lives, but also were finding only additional suffering in their engagement with a system in which the richness of their lives was reduced to a diagnosis and the treatments were imposed by experts. The statement formulated by Jon Kabat-Zinn, developer of Mindfulness-Based Stress Reduction (MBSR), "as long as you are breathing, there is more right with you than there is wrong, no matter how ill or hopeless you may feel"(1990, p.2), had a resonance that suggested the radical potential in both practice communities for me. Here was the sound of freedom from imposed identities, from imposed interventions, from surveillance by experts, from surveillance by oneself, and freedom to discover for oneself and with caring others how to work with and around one's unique suffering to live the fullest life possible.

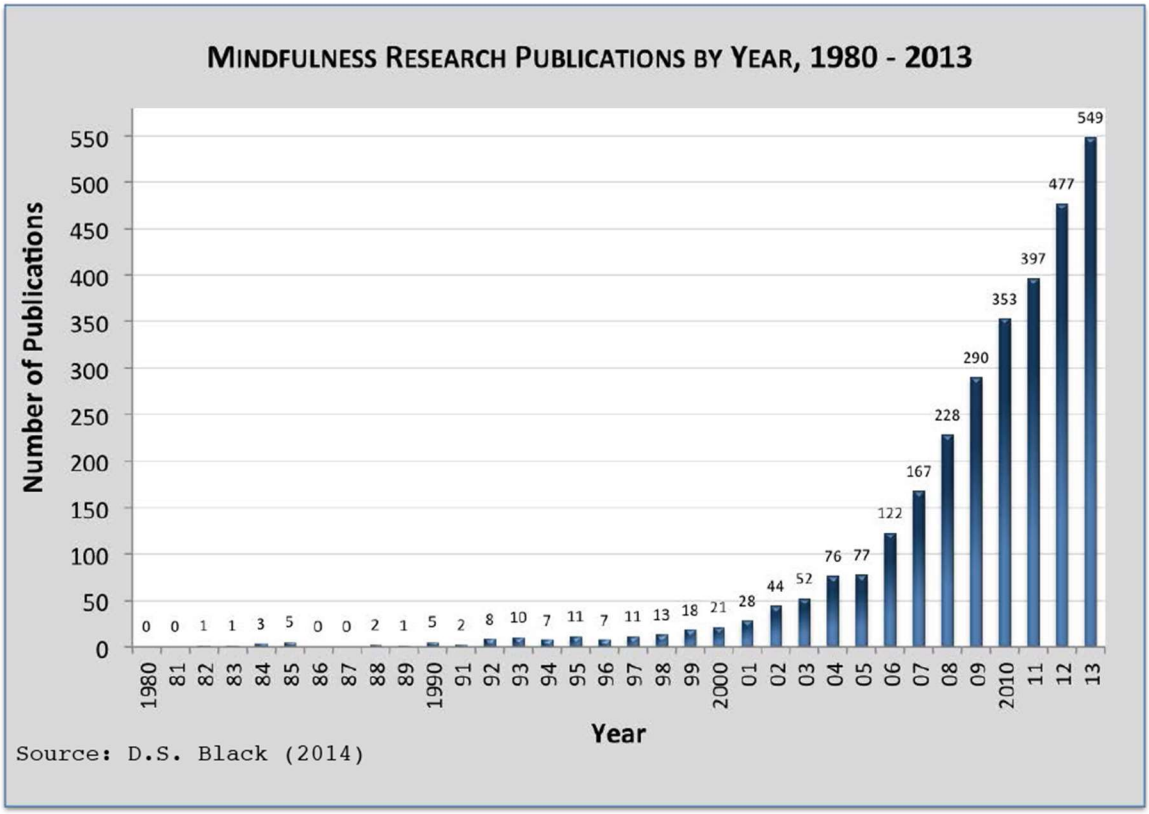
Through the intervening years, I've been excited to watch the social constructionist community live robustly, albeit incrementally, into its radical potential. At the same time, I've seen the mindfulness-based community grow dramatically on the world stage, both within and far beyond its original health care context, although not exactly in the direction of its radical potential.

It's bittersweet. I'm ambivalent.

The very things that have propelled the growth of mindfulness-based interventions have focused its discourse in some very limiting ways. These limitations are, to my mind, mis-directions required in the quest to achieve an unassailable scientific evidence base, to legitimize meditation in the discourse of health care. To achieve this, the Mindfulness-Based Interventions (MBIs) have ignored their historical position, substituted individualism for their inherent interdependence, accepted current diagnostic categories despite their holistic nature, emphasized an "inner dimension" in a practice that is actually developed in relationship, and defined mindfulness in a way that ignores its dynamic potential. I invite you to think this through with me. Once we've considered what limits the radical potential, we can turn to a different description that I think (and you shall judge) positions the mindfulness-based interventions as a direct challenge to the status quo mental health system.

**Research and its Mis-directions**

The drive for evidence-based intervention status has had a profound effect on the growth of the MBIs. MBSR was developed in 1979 as what became an eight-week course of training that includes a range of meditative practices, education in stress physiology and interpersonal communication, and other modules considered of value for the target audience. For its first couple of decades—the first study was published by Kabat-Zinn in 1982—MBSR was a barely known niche approach in mind-body medicine. Its character reflected the radical potential that attracted me. It was delivered in a group context, as education rather than therapy (sometimes delivered by medical or mental health experts, sometimes not), for a heterogeneous population—with the emphasis on "what's right with you."




*Black, D.S. (2014). Mindfulness-based interventions: An antidote to suffering in the context of substance use, misuse, and addiction. Substance Use & Misuse, 49(5):487.*
**Data obtained from a search for "mindfulness" in the Web of Science database. Search limited to research-related articles. Book-related material excluded.*

**The drive for evidence-based intervention status:** In the chart here, maybe you notice a change of pace at around the turn of the century. Roughly, this marks the development of Mindfulness-Based Cognitive Therapy drawn from the MBSR curriculum, joining mindfulness to the dominant discourse in clinical and academic psychology (Segal, Williams & Teasdale, 2002). The success of MBCT in reducing relapse in patients with three prior incidents of major depressive disorder was dramatic, stimulating much further research and the availability of significant government funding in its wake. Researching an appealing, safe, inexpensive, and effective intervention can

make academic careers. And has. And has expanded the applicability of the basic approach for medical and mental health conditions to include anything that the reduction of anxiety (sympathetic nervous system activation) might help: from patients with spine surgery pain to overweight adolescents, to name just two of the 10 studies funded by the US National Institutes of Health in April of this year (clinicaltrials.gov).

Yet, despite the triumphalism of the MBI community and allies in the worlds of education and business, objective looks at the scientific evidence so far don't live up to the hype. A review article in *JAMA* noted that "We found no evidence that meditation programs were better than any active treatment (i.e., drugs, exercise, and other behavioral therapies)," but that "Clinicians should be aware that meditation programs can result in small to moderate reductions of multiple negative dimensions of psychological stress. Thus, clinicians should be prepared to talk with their patients about the role that a meditation program could have in addressing psychological stress" (Goyal, *et al*., 2014). The price for playing the evidence-based game, to get the funding for all the randomized controlled trials that are required to legitimize the MBIs in the eyes of science, has been the adoption of views that support the necessary research at the expense of the unique character of mindfulness-based group interventions.

**Disavowing history:** For starters, the long history of meditation in psychotherapy (McCown & Micozzi, 2012) has been all but ignored in the literature because the discourse of legitimation was so different in the late Nineteenth and most of the Twentieth Century, when psychodynamic and humanistic approaches were influenced by meditative practices from Asian religions. Despite Freud's denial of a personal capacity for meditation, his description of the analyst's stance as "evenly suspended attention" is not a bad definition of mindfulness, while Jung had a daily yoga practice for decades, and Eric Fromm and Fritz Perls were greatly influenced by the insights of Zen Buddhism. Popularization of the term "mindfulness," which was extremely uncommon before 1990, further emphasized an historical break and the establishment of something new. Important historical insights were lost, but mindfulness didn't have to drag along that unflattering baggage of de-legitimized approaches. Contrariwise, with the success of MBCT, mindfulness could claim and pull along (or be pushed by!) the enormous evidence base accrued by the dominant cognitive and behavioral therapy interventions.

**Disavowing the group:** Despite the group composition of the MBIs, the research focuses exclusively on individual outcomes. Participants in the group are seen through research as bounded beings (to use Gergen's phrase), responding to the presumptive active ingredient of mindfulness meditation (which I'll come back to in a minute). The context of the class, presence of others, and the actions and presence of the teacher are factored out or dismissed. In fact, it took thirty years for anyone to directly study the effect of the group on individual outcomes (Imel, et al., 2008). Factoring out all other potential confounds (including the teacher), the study found that the influence of the group accounted for 7-percent of the variability in outcome. That's quite large; for perspective, in individual psychotherapy, the therapeutic alliance accounts for 5-percent of variability. That amazing finding went precisely nowhere: no further studies, and only the slightest of ripples in the theoretical literature. Effect of the teacher has not been studied empirically to my knowledge.

**Buying in to diagnostic categories:** Of course, the research has used the diagnostic categories of the DSM, in order to obtain funding. Now the US National Institutes of Mental Health have decided to bypass DSM-5 categories and focus on genetic, neurobiological, behavioral, environmental, and experiential factors—with emphasis that "Mental illnesses are now studied as brain disorders, specifically as disorders of brain circuits" (NIMH, 2008). So, MBI research will continue to locate problems inside individuals and take pictures of what's inside their heads as evidence. This discourse of individualism, of course, contrasts sharply with the MBI community's embrace of interdependence, as promoted in Buddhist modernism: the world as an interrelated web of processes in which a sacred balance must be obtained.

**Pressing agendas:** Individualist descriptions of pathology and its possible alleviation through mindfulness meditation support and are reflected in the MBI's emphasis on "personal practice" in which one turns "within" to find strength and peace—consider the names of mindfulness programs such as "Inward Bound" or Google's "Search Inside Yourself." This view reflects a basic assumption in MBI research that it is the meditation practice, and, particularly, compliance with the assigned daily homework, that is the "active ingredient" in the MBIs. This is despite the fact that MBI curricula are packed with a wide range of educational, participatory, and interpersonally experienced activities, offering many candidates for active ingredients. Thus, research has attempted to correlate amount of practice to outcome—so far with ambiguous results. Just as one way of parsing this, the original MBSR curriculum asked for 45 minutes per day of homework practice, with 45-minute recordings to support it. However, actual measures of compliance with these demands show significantly less practice being done, roughly 20 minutes (Carmody & Baer, 2008), and MBSR courses with much shorter class times and much shorter meditation homework have delivered outcomes consistent with those of the most strict version (e.g., Chang et al., 2008; Reibel et al., 2001; Jain, et al, 2007). It has been difficult to establish correlations of amounts and regularity of practice with improvements in outcome (Vettese, et al., 2009); what is more, any such correlations may be as indicative of participants' existential commitment to the ideals of the course as of the action of the practice in effecting change.

**Mis-directing definitions:** My final example of misdirection in research (patience! I'll get to the positive story very soon) involves the central question, "What is mindfulness?" Not only has the community had difficulty showing that meditation practice is the key, but it has expended vast amounts of energy in assaying definitions that can be easily operationalized for research. Philosophers and religion scholars, as well as scientists, have been pressed into service, dancing around definitions rooted in Buddhist psychology and practice, often involving philological studies of Pali and Sanskrit terms, such as *sati* and *smrti,* and generating far more heat than light: expanding, for example, to fill a special issue of the journal *Contemporary Buddhism.*

The mis-direction is obvious to those of us in the social construction community. Wittgenstein's (1953) concept of a "language game" comes to bear. A word comes into its meaning through its use within a form of life—a community. And the most important community in this case is not the community of MBI scholars and researchers, but rather the community of the MBI classroom, of the teachers and participants. The meaning of the term mindfulness that matters is not what the teacher says it is, imposing something

from above that will remain fixed; rather, it is what the teacher and participants create while learning what they learn together. Mindfulness, both as word and as performance, is *co-created* within the relationships of the classroom; it is unique in each class, and constantly changes and evolves.

So, mindfulness is not most usefully thought of as a skill to be taught and learned. It is better considered as identical with its pedagogy, which is a relational process. It is growing into a different way of being, with others.

**A New View from the Pedagogy**

The MBIs don't need to change all that much, in order to recover their radical potential. I'm most interested in re-describing them from a relational viewpoint, and I hope you'll bear with the clumsy approach I feel compelled to take. The character of the experience in an MBI group is difficult to capture, so I've been collecting ways of speaking about it that reflect different facets. I think of myself as a *bricoleur*. I'm from Philadelphia, home to the Magic Gardens of Isaiah Zagar on South Street, which are wonderful examples of *bricolage*. He uses what is at hand, whole and broken glass and pottery, old bicycles, dolls, broken mirrors— as you walk by (or better, walk through) there's dazzle and shine. The artwork is more than the sum of its parts, but there is no settled whole; there's somehow more clarity and more mystery at the same time.





I'm going to offer you thirteen different scavenged pieces—shiny, colorful, solid, precariously balanced—from my ever-growing collection. Maybe you can be patient and let them build towards something, or maybe you can appreciate each for its own sake. Just see what happens.

The first one, and there can be many like it, as these are the mirror shards in the mural, is a scene from an MBSR group:

**An MBI class scene:** *Participants are talking in dyads about their experiences with the sitting meditation we've just completed. Their low voices are mostly murmuring. As I look out over the room, their engagement is palpable. And varied. I absorb, in a slow sweep of my head, locked eyes and twinned smiles, closed-eye pauses and motionless listening, sadness and solicitude, the leaning-in of quiet laughter, a tentative touch of hand to hand. My own experience of the moment is like that of one of the angels in Wim Wenders' film* Wings of Desire*, perched above the city, listening, watching, simply present. I suspect there will be much to be said in the big group. I say, "Finishing up your conversations and coming back into our circle."*

*They acquiesce slowly, but the dyads part and turn, participants pulling chairs into something like the circle in which we began the evening. They are primed and rehearsed, in a way, for this question: "So, what did you discover that you'd like to talk about?" There is quiet for some time. I drop my shoulders and lean back, not forward. There is nothing now for me to do or say.*

*"My leg has just been agony for me," Susan says quietly. She has spoken of this before, and the surgeries she has had, and the surgery that is scheduled for the last week of our class. "This pain is just too much sometimes, and I can't just be with it, let alone be friendly. I can distract myself with the breath sometimes—that worked a little tonight—but I don't think I can be mindful at all."*

*"Susan?" I ask. "How is the pain right now?" After a moment of checking in, she finds, "It's not screaming, but it's definitely there."*

*"Would you be willing to see if you can work with this mindfully? At least to explore the possibility for a moment or two?" No pressure from me, as she knows. She's said "no" before.*

*"I can try," she says.*

*I look around at the now serious faces, moved by Susan's disclosure. I say, "This is how it is for all of us at one time or another—maybe there's some kind of pain now for you too. Seeing if you can be present to what happens for you, as we make this exploration." I turn to Susan. "So, just bringing your attention to your breathing right now."*

*Her hand goes immediately to the top of her chest, her throat. "There's a tightness, a pain, here in my chest."*

*"OK, so can you bring your attention to that feeling? Does it help to hold your hand there?"*

*"Yeah, it helps. A little comfort for myself… sympathy even." Quiet breathing. The room quiet. Then, "I think I'm just really tired of living with the pain I'm going through, but I don't want to give in."*

*"What you're doing right now, turning towards your experience, isn't giving in, at all, Susan. It's sort of the opposite. It's showing up. It's being right there in your life."*

*"OK." Very tentatively.*

*"So, do you think you can check in, just for a moment now, to how it is in your body?"*

*"In my leg, it's kind of like ginger ale bubbles breaking. But sometimes it's like it's on fire—too much to stand."*

*"But not right now? It's not too much to stand now?"*

*"It's just that it's so constant, always there. That's why I can't stand it."*

*"So that's the story, the thoughts that you have? 'This never goes away.' 'I'm always in agony.' 'I can't stand this'?"*

*"That's my story," trying a wry smile.*

*"If you turn your attention towards the leg in this moment, what do you find?"*

*A pause to study. "It's still fizzing. And that's scary. I'm afraid it will get worse."*

*"So, fear is here now?" Susan nods. "And where does the fear show up in your body?"*

*"Everything tenses up… back, shoulders, neck… look, I'm making fists."*

*"And the leg?"*

*"Faster fizzing… hotter… It's getting worse." Pitch and volume rising, "I don't want this… It's not fair…" Her face is down and her fists are up.*

*"I get that. And so do other folks. I see the nods and expressions around the circle." She looks up and around.*

*"Susan, I get that don't want this pain, that you want to push it away. But right now, it's here, and so are you. Is it maybe possible to try a new approach. Not giving in to it, but opening up little bit," gesturing with thumb and forefinger together, "to the sensations in your leg, to your fear, your anger, the tension in your body—to whatever it is that's here." I look around the circle, "And whatever is here for you, as well, is your own undeniable experience. How are you meeting it? Pushing away? Or opening up? Either way, what is it like for you?"*

*We sit quietly together for a minute, 60 seconds or so, which is a long time.*

*I turn towards Susan. "What are you noticing?"*

*"Well, I unclenched my fists," a little sniffing laugh. "That's different… There's less tension in my shoulders and back… I'm aware of my legs… I'm aware that I could tell myself how awful it might be."*

*"But you're not doing that?"*

*"No, I'm just noticing that thought. There's more going on somehow than just me and the pain. I feel, I don't know, bigger."*

* * *

And so I now ask you, in this room, in this moment, how is it for you to have heard that story? What is it like in your body?

Maybe we can talk a little. I think there's time….

* * *

Now, maybe we know a little more of what we're talking about. In an MBI group, the practice (and the challenge) is to be with/in whatever is happening in the present moment. And this all happens in relationship.

**Aristotle on friendship:** As our next piece for this bricolage, there's Aristotle's description of friendship, the most important of virtues. In the *Nichomachean Ethics,* he's concerned with what will hold the *Polis* together, with how we can be together. The image he uses is one of travelers—say on a sea voyage—vulnerable to storm and accident and facing the vastness of nature and the journey. (A little like the MBI class, no?) He notices how they hold each other as "close" and "dear" in their shared passage. He finds the solidarity that arises, the sympathy and like-mindedness, to be like the excellent form of friendship.

In excellent friendship (*teleia philia*), the friends are not oriented towards each other, looking to gain something that the other owns, but rather, they share a common orientation towards excellence, towards the good. "Perfect friendship is between human beings who are good and similar with respect to virtue; for, insofar as they are good, it is in a similar manner that they wish each other the good, and such human beings are good in themselves" (1156b7-10). In the case of the MBI group, the good we are oriented towards is the practice of mindfulness, the turning towards and being with/in the experience of the present moment. The class is a gathering of friends.

**A "gathered" meeting:** That suggests another shard for the bricolage. In the Society of Friends (Quakers), the meeting for worship is a silent *waiting* for the moving of the spirit, in the older usage of waiting as attending to a royal personage: being still and silent yet ready to respond instantly to any need or asking. In this practice, special times come, when, "a blanket of divine covering comes over the room, a stillness that can be felt is over all, and the worshippers are gathered into a unity and synthesis of life which is amazing indeed. A quickening presence pervades us…and awakens us in depths that had before been slumbering. The burning bush has been kindled in our midst, and we stand together on holy ground" (Kelly, 1947, p3).

OK. Maybe now it's time to raid the stores of the science folks for some bits and pieces from another perspective.

**A "scientific: description:** Key to a scientific description is the action of mirror neurons, which help to sense, represent, and track another's actions and intentions (Gallese, et al., 1996). Humans attune and resonate with each other. We're empathic. (Who knew?)

When you witness another's pain or joy, you feel it in your body. The neurobiological account tells us that what you see activates the brain's "resonance circuit," as Daniel Siegel (2007) calls it. Described by Carr, et al., in 2003, this circuit carries the first sense of the other's movement or expression from the mirror neuron system (where you "try it on"), to the superior temporal cortex to process how it feels in the body, then through the insula to the limbic system to get the emotional content, and back through the insula to the prefrontal cortex define how the other feels.

Siegel (2007) suggests that this interpersonal resonance circuit also works *intra*-personally for the person practicing meditation. The prefrontal cortex is active, so it down-regulates the limbic system, particularly the amygdalae, thus reducing negative affect such as anxiety and fear (Creswell, et al., 2007; Lieberman, et al., 2007).

Let's move to an MBI class that is in meditation, potentially resonating intra-personally (and perhaps interpersonally) and feeling some equanimity or even something akin to "happiness." In fact, evidence is emerging that observers perceive meditators as happier than other folks (Choi, Karremans, & Barendgret, 2011).

One more move: Stephen Porges's (2011) "polyvagal theory" of regulation of the autonomic nervous system (ANS). This theory is based on the evolution of the ANS, particularly the vagus nerves, in vertebrates. In mammals, there are three phylogenetic stages expressed as three subsystems, with each subsystem linked to three behavioral strategies for adapting to catastrophic and life-threatening situations, or those just dangerous or challenging, and—this is surprising, perhaps—to situations of safety and caring. Respectively, the strategies are *freeze*, *fight/flight*, and what Porges calls *social engagement*—which is the surprising one that we most care about. It is associated with the newest vagus, which responds to a sense of safety in the environment by slowing the heart rate, inhibiting fight/flight, and—here's the social part—regulating the muscles of the face and head for optimal meeting with others. In detail, the eyes open wider to see others better, the inner ear tunes to the range of the human voice, the muscles of the face and neck are toned to produce even the most subtle expressions and gestures, while the muscles of speech are toned perfectly as well. Best of all, oxytocin (the "love" hormone of birthing, nursing, and pair bonding) is released. In the group, then, when we discover familiar or friendly faces, voices, gestures, and postures, which may happen at the end of

a meditation session, the social engagement response may begin for some, ushering in positive changes of expression and posture. The sense of calm and safety may then travel around the group in a self-reinforcing way. The result is the possibility for participants to use their sense of being regulated, their sense of safety, to turn towards and be with/in whatever experience is arising in the present moment.

**Shotter's "joint action":** John Shotter has been concerned for decades with the question of *how* we create the ways we are together. He locates these processes in the continuous hubbub, the chaotic flow of moment-by-moment life in which we are embedded. Shotter refers to this in the term "joint action," which he has elaborated over time, particularly through insights of Bahktin and Merleau-Ponty (1984, 1993, 2008, 2011). Bahktin brings a dialogical structure in which words spoken to and from one another are responses to the dialogue partner and the moment of speaking—what has come before and what is needed or anticipated (Shotter, 2008). Merleau-Ponty thickens the description further, as a structure that is chiasmically intertwined. Our "making sense" of the moment is not intellectual, not cognitive, but rather, bodily. Our living organism integrates, somehow, our perceptions and action potentials, so that we maintain our orientation in the present (Shotter, 2011). Joint action brings us into being; through participation in joint action, each of us "will have come to embody different ways of perceiving, thinking, talking, acting, and valuing" (Shotter, 2008, p.37). Each will be a new *kind* of person: through joint action with music teachers and a band, one may be a musician, or after the joint action of an MBI course, one may be a meditator and/or a less-reactive, more accepting parent, spouse, or leader.

Shotter (2008) has identified at least three features of joint action that are important for a description of an MBI group. First, joint action does not derive from the intentions of particular participants, but rather in the exchanges among them. Joint action cannot be predicted, so participants find themselves in a "situation" that is specifically their own. Second, the situation points beyond itself. It is open to and actually invites actions by the participants. Third, participant actions establish the situation as a particular world or reality that is dialogically structured, built through spoken and unspoken responses and anticipations, sensuous perceptual experiences, and feelings. Further, these unique features cannot be located in particular participants, rather they are evenly distributed among all, and are found neither inside nor outside the participants, but rather in the shared space.

Consider the scene with Susan, or even the scene in which I read the scene with Susan to you all, in this description from Shotter:

> *We cannot prevent ourselves from being spontaneously responsive to events occurring in our surroundings; we react to them and to each other's activities bodily, in a "living" way, spontaneously, without our having first "to work out" how to respond to them. But more than simply responding to each other in a sequential manner—that is, instead of one person first acting individually and independently of another, and then the second also by acting individually and independently of the first in his/her reply—the fact is that in such a sphere of spontaneously responsive dialogically structured activity as this, we all act jointly as a collective we (2011, p58).*

**Breaking eggs together:** Anthropologist Tim Ingold (2008) rebels against the term "socialization" in describing teaching and learning. He insists that learning does not change the biological raw material of a person to some ideal finished state, but rather that the learning never ends and remains biological and social, always. He describes this ongoing, situated, relational process as *enskillment*. Consider the skill of making an omelette. One learns the feel for it under guidance from someone who knows the process tacitly. Every egg is unique, and the way to crack it is therefore unique; the cook's hand can help you find the way. And you won't find it in a place divorced from cooking; you naturally learn in a kitchen. Your knowledge does not become implanted on your inside from outside. You grow into successfully cooking omelettes in the kitchen with the cook; and your knowledge belongs to the whole system—the specific space, materials, and relationships. As Ingold puts it (2008, p.116) "you only get an omelette from a cook-in-the-kitchen." Ingold might be considering an MBI group here (2008, p.117): "The minds of novices are not so much 'filled up' with the stuff of culture, as 'tuned up' to the particular circumstances of the environment. It is this tuning that enables them to make their way in the world."

**So much more:** Of course, Vygotsky's (1978) *Zone of Proximal Development* is sparkling in the grout of this bricolage. As is Buber's possibility of silent *dialogue* (1947). As are van Gennep's *rites of passage* (1909) and Victor Turner's *liminality* and *communitas* (1969).

**A Buddhist aside:** And there's a wonderful little vignette of early Buddhist life, in which Ananda, one of the Buddha's chief disciples, sits down next to the man himself and says something like, "This is half of the holy life, lord; admirable friendship, admirable companionship, admirable camaraderie." The Buddha replies, "Don't say that, Ananda. Don't say that. Admirable friendship, admirable companionship, admirable camaraderie is actually the *whole* of the holy life. When a monk has admirable people as friends, companions, and comrades, he can be expected to develop and pursue the noble eightfold path"*(SN 45.2).*

**Gergen's relational being:** Although I've not been applying the terms, Ken Gergen's (2009) discourse of *relational being* describes the two parts of this talk. The individualist view with which we began is of *bounded being*, which describes the discourse of autonomous selves setting intentions and taking voluntary actions, as well as the idea of causality that allows us to explain and then research ways that people are broken on the inside, and ways to fix them by putting something else in there—a virtue, a belief, a pill, even a practice. *Relational being* is unbounded. It's not that one acts from some inner, pre-existing me (which can be broken and fixed on the inside), but rather that there are not separate identities within relationships, but that which can be labeled as me is co-created in the moment.

The language required for this concept is not readily available, because the language that we've inherited is within the bounded discourse. To gain some clarity, Gergen uses the idea of *confluence,* in which "participants" (language limitation warning!) are mutually defined through the unfolding form of life in the moment. A session of formal meditation in an MBI class, for example, mutually defines meditators who are sitting still and quiet, and a teacher who is speaking words of "guidance." As the meditation confluence ends, a new confluence may form as meditators become dyad

partners and speak aloud to one another, in the same spacious, curious, and non-judging manner that the teacher may inquire of participants' in-the-moment experiences in the confluence of the plenary group that will follow. There is no causality, nothing forces teacher or participants to do what they do; there is likewise no agency, no force inside compels them. What happens next in the class issues from the confluence of relationships in which, say, a plenary dialogue makes sense.

As in all these other glittering shards, there is a sense of continual, gentle, bottom-up transformation. As Gergen has it, in participating in the confluence, we gain new possibilities: "In sum, all meaning/full relationships leave us with another's way of being, a self that we become through the relationship, and a choreography of co-action. From these three sources, we emerge with enormous possibilities for being" (2009, p.137).

**The Ethical Space of the MBIs**

Maybe in a description of the relational situation, the joint action, the confluence of the MBIs, you can get a sense of the qualities in which the class is steeping. Through analysis of the pedagogy of the MBIs required for writing the first textbook and developing curricula for training MBI teachers, I have attempted to identify the key qualities of what I call the *ethical space* of mindfulness in clinical practice, in both the actions of teaching and the unspoken framing of the space (McCown, 2013). I've suggested a model that has seven key qualities, distributed across three dimensions. Also, it should be noted here that the space of which I speak is actual architectural volume, not an abstraction. As you'll see, MBIs are characterized by attention to the corporeal experience that unfolds for the group moment to moment during the class.

In the *doing* dimension, there are three qualities that define the work of the MBI class. These are the qualities that are imbued in the joint action, in the confluence, of the ongoing attempt to stay with/in the experience of the present moment.

*Corporeality* is the quality that foregrounds the experience of the body, which is quickly recognized as different from typical modes of investigation in mental health interventions. Mindfulness meditation is an embodied practice, founded on sensation—particularly of the breath moving in the body. This sense of embodiment brings the group into intimate contact with present moment experience, and helps make aesthetic and affective experiences available and tolerable for exploration and dialogue.

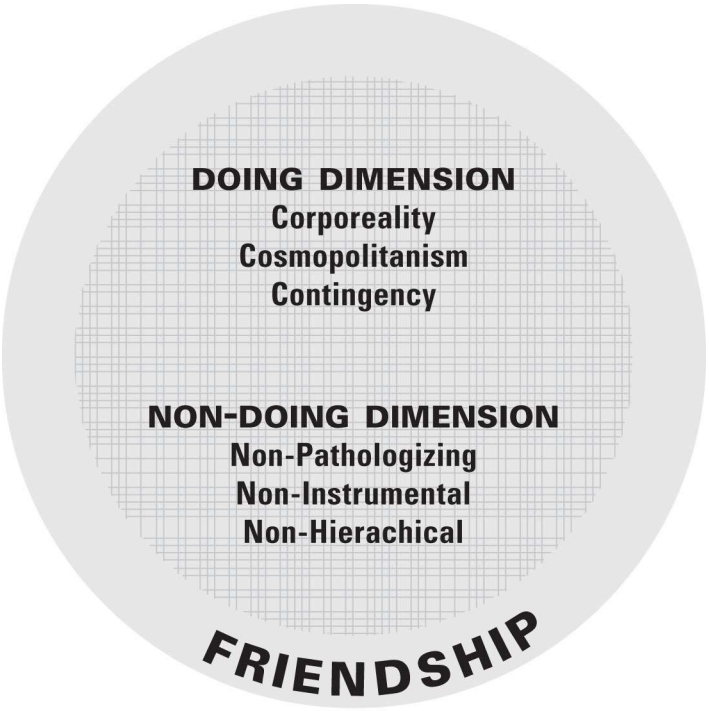
C*ontingency* is the quality that deconstructs these experiences, as the class tracks the arising and subsiding of sensations, affects, and thoughts. In seeing how sensations continually change and pass away; in encountering the relation of distressing (or desired) affect to passing sensations; and, finally by noticing the instability of thoughts, insight and meaning may arise.

*Cosmopolitanism* is the quality that holds the meaning. The term is meant to describe the acceptance of the meaning that arises in the moment, without a drive to abstract, reduce, or fit it into any system or set of values. Meaning, in other words, is revealed as contingent.

In the dimension of *non-doing*, it is easy to focus on the teacher as the actor establishing the qualities; yet, as I hope you've seen in our explorations of the pedagogy that is the practice, and in the illustration of dialogue with Susan, these qualities are actually inherent in the shape of the confluence.

*Non-pathologizing* refers to that defining perspective that "if you are breathing

there is more right with you than there is wrong." Ideally, no labels are invoked in the




*The interweaving of the doing and non-doing dimensions is ultimately infused with the quality of friendship, Aristotle's* teleia philia.

Dialogue that happens aloud, and there is a possibility to allow the unfinished dialogue called thinking to be deconstructed, and any pathologizing self-surveillance to be undermined as well.

*Non-hierarchical* refers beyond the teacher as well, to describe the group relationship in dialogue; no meaning can be imposed on the ever-changing experience, and no one is expert on the unfolding of the present moment. To phrase it in American vernacular speech, the teacher is as clueless as anyone else, and is simply interested in exploring *with/in* the confluence.

*Non-instrumental* is perhaps most important for this conference, yet it may be the most difficult to grasp of the qualities. The class does not practice the pedagogy of mindfulness in order to be changed or transformed in a particular way. We don't practice "because" or "in order to" but rather as exploration of the unknown of the present moment. Transformation is, however, the nature of the confluence—watching all contingent structures of sensation, affect, and thought deconstruct themselves as they unfold within the space and its associated qualities: always changing, eluding ultimate meaning, guided only by the unfolding relationships in the moment.

*Friendship* is not a dimension or a quality, rather it is the total character of the confluence of the MBIs. It is not "held" by the teacher in some way; there is no choice to be friendly; rather, friendship is a "possibility of being" that participants in MBIs steep in and may carry on and away from the confluence. Friendship is that which may be reflected in actions in relationships of other confluences, and even in relationships of unfinished dialogue.

**The Ambivalent Promise of the MBIs**

As I understood it, fifteen years ago, the strategies for research and growth of mindfulness in clinical practice in medicine and mental health care were necessary for success, yet also offered potential for doing *Aikido* on the system—joining with its energy and then using that to turn it. I imagined a rise in popularity and publicity for mindfulness approaches, which would help transform access, cost, and social implications of the alleviation of suffering, and would usher in a new era of ideographic forms of research—especially in mental health care.

Considering the engagement of the MBIs with the current system, I see no evidence of a shifting of weight or moving of a hip—no *Aikido* moves are on the way from the academicians and clinicians from the established professions. They are doing well by making careers in what I consider a highly compromised way. They continue to ignore their historical position, to substitute individualism for interdependence, to leverage current diagnostic categories and the drive for neuro-bio-medicalization, to emphasize an "inner dimension" in a practice that is *obviously* (!) developed in relationship, and to define mindfulness in ways that ignore its dynamic potential.

I would be more pessimistic than ambivalent about the radical potential of the MBIs, were it not for one long-standing tension within the community. While researchers hold the compromised positions that I have outlined, the folks involved in teaching and teacher training struggle to work effectively under those conditions. Colleagues and I have set this problem before the community (e.g., McCown & Wiley, 2009, 2008), unfortunately to very little effect. Nevertheless, that there is a gap between what teachers experience in the classroom and what researchers are looking for gives me some hope.

If teachers can begin to close the gap, maybe we can do some *Aikido* on the rest of our own community, resulting in small shifts in the larger system. In the UK, where the demand for teachers is very high, due to the NICE guidelines promoting MBCT and MBSR a preferred treatments, the teaching community has chosen to assume the identity "teacher" rather than "therapist" (Crane, et al., 2010). That's something. And in many areas of the world, those teaching the MBIs are not professional clinicians, but are highly experienced (steeped) in the teaching. Maybe roles and identities are a little bit shaky. And that's something more.

If we teachers took our position seriously, or, better, took the ethical space seriously, we might subvert the clinical identity of the MBIs all together, and just work on ways of helping others help each other. Being together might then become being a *together,* hearing together as a noun, a name for joint action, confluence.

What would it look like if this could be played out? The first dramatic change, I believe, would be a solution to a rarely discussed problem in the MBIs.

As folks complete their group experience, they are mostly cut loose to practice alone, since that is the typical understanding of what happens; they learned the skill, and now they can go off and do it for themselves, by themselves—with, in some programs, opportunities to be a together monthly for an hour or so.

However, if the context were to change from clinical to something more accessible—not reinforcing diagnoses and histories, but instead celebrating common human brokenness and potentials—gatherings might look like part of a secular, spiritual movement. I think here of the "emerging church" as imagined by activists such as Peter Rollins (e.g., 2012) with an added depth of silent contemplative practice. Certainly, such

groups could be ongoing, offering a together experience over years rather than weeks, greater potential for accruing possibilities to live into, and, I imagine, a community in which the teacher really could work herself quickly out of a job—the ultimate goal. Such an approach would be more accessible: it could more fully live out the qualities of the MBIs to be non-pathologizing, non-hierarchical, and non-instrumental.

I love the way that Rollins uses public spaces—public bars—for meetings of his IKON groups, making the coming together non-threatening and highly accessible. Perhaps a bar is a not the best venue for a more silent contemplative group, but a coffee shop or a gallery space would accomplish much the same effect. Just people, together, offering our capacity for being to each other, steeping (maybe a tea shop is a good place, too!) in rich possibilities.

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